

CITATION: Baines v. Hehar, 2011 ONSC 1842
COURT FILE NO.: 06-CV-323766
DATE: 20110323

SUPERIOR COURT OF JUSTICE - ONTARIO

RE: ELEANOR BAINES

Plaintiff

AND:

NAVDEEP SINGH HEHAR and MANMOHAN S. HEHAR

Defendants

BEFORE: JUSTICE MOORE

COUNSEL: *Eleanor Baines*, In Person

Alexander Lempp and Jeff Booth, for the Defendants

HEARD: March 21, 2011

ENDORSEMENT

[1] Ms Baines was involved in a motor vehicle accident on 26 January 2000. She brings this action for damages arising from that accident and asserts that her ongoing limitations are permanent serious impairments of important physical, mental or psychological function, within the meaning of those terms as used in the *Insurance Act*.

[2] During the collision in question here, Ms Baines recalls striking her head on a fiberglass panel near the windshield of her car. She was dazed but not unconscious. Her recall for the circumstances surrounding the accident is intact.

[3] This action proceeded to trial before a jury on 3 March 2011, over 11 years after the accident. The trial continued through 10 days, during which Ms Baines self represented her interests and insisted, both personally in the evidence she gave and through the evidence she lead from other witnesses and in her cross examination of defence witnesses, that she suffered a brain injury and that the universe of complaints that she has voiced over the years since the accident confirm the seriousness and permanence of her limitations.

[4] Following the closings and jury charge, while the jury was deliberating, defence counsel indicated that this motion would be brought to establish whether the plaintiff's claims are barred by the applicable threshold established under Bill 59. Given that Ms Baines was wholly

unfamiliar with the issues raised by this motion she sought and received time to prepare and respond. In the interim, the jury returned its verdict.

[5] My task is to answer questions that were not specifically asked of the jury. I will assess the plaintiff's entitlement to non pecuniary damages despite rather than because of what I might suspect motivated the jury in answering its jury questions in the manner it did.

Background

[6] Ms Baines testified that she was a well woman, a high functioning person before the accident. Ms. Baines began her evidence by speaking about her background. For example, she testified that she was born in Toronto in 1956 and that she became a single mother at age 21 and that her only child, her daughter Aisha, is now 33 years of age. She has three grandchildren aged one, five and nine.

[7] Ms. Baines described terrible pain in her head and knee pain and other generalized pain immediately following the accident. She took between eight and 12 aspirin at the outset to mask her pain.

[8] Ms. Baines said that she tried to read all that she could about head injuries. She said she reads magazines and articles published by the Brain Injury Association of Ontario.

[9] She said that she was unable to retain information, had difficulties with forgetting, difficulties with her sleep and became agitated quickly. She found that she was unable to visit with her father as often or for as long as she did before the accident.

[10] Her goal had been to become qualified to obtain a teaching certificate and then to teach in an elementary school until age 70 by which time she would have amassed 15 years of service and a pension.

[11] She wanted to get married if the right man came along and she wanted to help to tend to her parents needs.

[12] Ms Baines described numerous ongoing symptoms. She had cold and flu symptoms and was often bedridden. She had bouts of blurry vision, chronic pain, clumsiness, rapid heartbeat, extreme insomnia and problems with weight gain.

[13] The majority of her symptoms occurred immediately after the accident and many of those have not improved since. She says that she was a high functioning person before the accident but now she cannot be that person. She has difficulty problem solving. She often needs to ask clarification in order to grasp new concepts. She says that she is unable to think creatively or to work with abstract thoughts. She is unable to express her thoughts without writing them down.

[14] Ms. Baines points out that her daughter and her grandchildren have not had the privilege of knowing her as the woman that she was before the accident.

[15] Ms. Baines spoke of having mental and physical fatigue; she testified that she has an overwhelming desire to lie down on the spot when she feels stressed.

[16] She believes that both her short-term and long-term memory has been impaired. She described how each day she has to "micromanage" tasks of her daily living activities. She described leaving pots on the stove, of finding unexplained bruising, she forgets people's names, people that she has known for a while, and she has locked herself out of her car.

[17] Ms. Baines believes that she may be a danger to herself or others. She is afraid of driving through stop signs and says that she does not drive when she is tired, such as when she has not had her afternoon nap.

[18] She believes that she is now "severely" accident prone. She gave an example of one time when she was intending to sit on a couch but fell from a standing position on to the couch. She forgot to crouch down. She hurt her back.

[19] She described difficulties with stairs. She says that she can forget a stair. She says she falls regularly.

[20] She kept notes on all of her problems so that she can remember them. They include:

- Headaches, that are not too bad because she paces herself and has an afternoon nap.
- Frequent urination at night is a problem
- She complains of loose bowels, something that was not a problem before the accident.
- She had experiences pent-up energy in her arms and legs
- She has pain in both of her legs and thighs
- She is pain in the right knee which has recently become "really bad". She has stiffness in her back and arms
- She loses her breath on stairs and on walking any distance
- She has difficulty controlling her anger when in stressful situations.

[21] She believes that her future marriage possibilities are limited. She does not think she would make a competent wife. She has generalized feelings of inadequacy.

[22] Recreational, social, spiritual, cooking, athletics are among the activities that she feels limited in and this is very frustrating for her.

[23] Before this accident, she said that she functioned well including at work. After the accident, however, she demonstrated a "high degree of incompetence" and had difficulty understanding instructions. She says that between 70 and 75% of the time her memory fails her.

[24] Ms. Baines agreed in cross-examination that fatigue and cognitive complaints [memory and concentration problems] are the main issues for her. Her physical injuries are now minimal because she does not do a lot. When she is active, however, she does experience "a lot of pain".

[25] Ms. Baines can drive and can, although seldom does, use the TTC. At home she can do chores such as cooking, washing dishes and laundry but she paces herself. She buys groceries in small quantities.

[26] Ms. Baines lives alone. Her mother had lived with her for four years before the accident until she went into a nursing home, in the month before the accident. At that time, Ms. Baines' daughter was 22 and was still living at home with her.

[27] Ms Baines confirmed that she has three other lawsuits currently outstanding before Ontario courts. One arises from a denial of CPP benefits and there is a judicial review hearing scheduled for late March of this year. Another is a claim for \$250,000 brought against her former lawyers and there is a claim brought against her insurance company in connection with accident benefits claims. She is self represented in all three of these matters.

[28] In preparation for the many matters that Ms. Baines focuses her attention upon, she works from a computer at home and, as well, has done research at the Great Library of the Law Society and at the University of Toronto law library.

[29] Ms. Baines testified that she spends considerable time putting her thoughts together and often her writings deal with similar issues. As well, she does have typing skills and some computer software skills. Finally, she points out that she has no one looking over her shoulder ready to fire her if she makes mistakes when she prepares documents for use in her litigation. Nevertheless, she described her litigation as her job and she works long and hard at it.

[30] The court heard evidence from Ms Baines' family and friends. Her brother, Frank Baines, testified that he "definitely" noticed changes in his sister. Before the accident she was extremely dedicated to her mother, a big responsibility, in addition to which she looked after herself and her apartment. After the accident, however, she did not appear to him to have the same energy and was not the same person at all. Mr Baines was a particularly impressive witness but he sees his sister only occasionally.

[31] Eileen Diane Baines is the plaintiff's younger sister. She spoke of her relationship with her sister before the accident in question and described Ms. Baines as a vibrant woman, someone who she looked up to. Theirs was a close relationship throughout their life. She recalled that Ms. Baines was trying to finish her school and to raise her daughter. She added that Ms. Baines enjoyed "just generally a good life".

[32] This witness noticed that after the accident her sister was:

- tired all the time
- forgetting things
- the conversations were scattered

- she lost her once in a shopping mall and the plaintiff did not recognize her sister when she found her
- the plaintiff would want to return home within about an hour because she was tired
- her thought processes were not the same as before
- I'm giving the plaintiff instructions, they would not come back the same as though the plaintiff had not heard or did not understand.

[33] Like her brother before her, this Ms Baines did well as a witness but she too sees her sister, the plaintiff, infrequently. Her evidence was about subjective complaints made in her presence. Neither she nor her brother sought medical or other assistance for their sister, to sort out what the complaints involved nor how they could be better managed.

[34] In cross examination, she admitted that her sister was active in visiting their parents after the accident; the number of visits per week varied but sometimes could be five or six visits per week.

[35] This Ms Baines testified that she was not aware of the plaintiff's involvement in other lawsuits. She admitted that the sisters still loved each other and that she came to court to support her sister and to give testimony favorable to her sister.

[36] Aisha Cynthia Baines is the daughter and only child of the plaintiff. She is now the mother of three children aged nine, five and one and she lives with those children only three buildings away from her mother in the St. Lawrence market district of Toronto.

[37] Aisha testified that she did not notice changes in her mother right away after the accident; this is difficult to reconcile with the plaintiff's evidence, particularly as Aisha lived at home with her mother at that time. The changes she eventually did notice were not apparent until they affected Aisha. She spoke of noticing memory problems and seeing her mother at home rather than at Jehovah's Witnesses meetings, for example. She recalled seeing bandages and noting that her mother had apparently hit her head on a cupboard door or bumped her leg. She noted that her mother often appeared to be tired and that her energy level seemed low and that she demonstrated clumsiness. These, she said, were some of the first signs.

[38] To Aisha's observation, the plaintiff is not now 100% the way she was before the accident in terms of her activity level. She plans now for activities during a one or two hour window. Aisha's children can't interact more with their grandmother and, Aisha feels, the children are shortchanged and have only known their grandmother as the grandmother who was always tired.

[39] All of this said, however, Aisha conceded that she regularly leaves her young children in the plaintiff's care for transport to and from schools and day care facilities and for babysitting. Aisha is comfortable in believing Ms Baines is fully capable of caring for the children, even in emergency situations, such as when her asthmatic son has an attack requiring medical or hospital care.

[40] Aisha has some but only limited knowledge of her mother's cases in litigation. Aisha has "seen her in action", by which she meant that she has seen her mother working on her cases at her mother's home. Clearly, Aisha has not seen her mother in action in this trial, a subject about which I will say more in these reasons.

[41] Ms Jaisaree appeared to be near tears when she volunteered that the accident has obviously changed the plaintiff's life. Ms. Baines has become a very different person in the view of this witness and the witness finds this to be very sad and she says it really bothers her to see Ms. Baines the way she now is.

[42] Of course, this witness travelled to Mexico with Ms Baines and spent many days together sightseeing and ministering to families there. There was no evidence that that trip was adversely impacted for either of them by Ms Baines' complaints.

[43] This witness was not aware of the number or nature of Ms Baines' several lawsuits. She was not present in the courtroom other than while giving her evidence. It is very difficult to reconcile her observations made on her few visits to the plaintiff's home over the years with other evidence she gave or to the observations of those of us who in fact did see Ms Baines conduct this trial.

[44] In short, the lay witnesses offered evidence to support the plaintiff, no doubt as a person and also as a plaintiff in this case. As to the former, their support may be laudable but as to the latter, their evidence is of little assistance on this motion.

[45] To an extent, each of the lay witnesses contradicted the evidence heard from the plaintiff. None of the lay witnesses could address the origin or cause of the plaintiff's subjective complaints and none were positioned, geographically or temporally, to speak to fluctuations in the plaintiff's functioning throughout the time since the accident or during the course of this litigation and trial.

Medical Evidence

[46] Dr. Mamelak is a psychiatrist who has been treating Ms Baines since 2000. He prepared a report detailing his treatment and opinions in 2002. He offered no opinion post dating that report.

[47] Dr. Mamelak believes that the referring general practitioner advised that Ms. Baines had suffered a concussion. He added that he accepts what the plaintiff told him about the accident and the symptoms that she reported. He said he has no reason not to.

[48] The list of symptoms that Ms. Baines complained of was one that Dr. Mamelak says he frequently sees in patients following upon car accidents, especially where one strikes the left side of the skull. Included in her complaints was that she was dazed for quite a while but did not lose consciousness and he said that this meets the definition of concussion. As well, she complained of headaches, neck pain, shoulder pain and some low back pain.

[49] Also, Ms. Baines complained that her concentration was very bad, that she had trouble reading and remembering things, that she had trouble sleeping, although that was getting better by the time he saw her and that she was tired during the day [he called this fatigue].

[50] She complained of a change in personality, that she was often irritable and angry and that she was withdrawn from social interactions.

[51] She complained of cognitive difficulties. She said she had difficulty with concentration, memory, multitasking and that she was distractible. The ordinary cognitive activities of daily living were apparently more difficult for her, he said.

[52] Dr. Mamelak advised that he had no reason to question the veracity of what he heard from Ms. Baines. He said that she could have taken it from a book.

[53] Dr. Mamelak was asked whether Ms. Baines was depressed. He said her condition fluctuated but "by and large" he did not see depression, as he understands it, as a major theme. He did notice anxiety and that Ms. Baines always seemed to look toward a gloomy future. She apparently felt that the accident had ruined all of her plans, hopes and goals, he said.

[54] Dr. Mamelak pointed out that pain is subjective and no one can feel anyone else's pain. This he described as "the dilemma" of psychiatry.

[55] Dr. Bail is a psychiatrist who has been in practice as such for 25 years.

[56] Dr. Bail interviewed Ms Baines and felt that she was evasive, guarded and uncooperative.

[57] In the course of taking her history, she was asked about her moods, as a focus on depression and anxiety issues. Ms. Baines said that she was okay when she was not under pressure or stress. She was not able to say how long she was okay but said that she was depressed sometimes but not all the time. She denied thoughts of death or suicidal ideation.

[58] Dr Bail performed a mental status examination on Ms Baines. During the course of this examination, the examiner studies the presentation of the patient, her flow of speech and assesses her mood. He analyzes whether her thought flow is coherent. He measures all mental parameters and this, he asserts, is a standard, procedure. Then he administers a series of tests such as the serial sevens test by which the person is asked to count backwards from off hundred by sevens, a measure of concentration.

[59] On this test, the plaintiff stuck with the process but answered with near misses in the numbers, which he said was evidence of someone who attempts to misrepresent.

[60] All of the mental status tests were described as very simple screening tests that people of almost any level of education can do, even children in grade 1. Dr. Bail stated that he has given these tests to thousands of people including people with limited English language skills.

[61] On the digit recall test, she was able to repeat three numbers but gave bizarre answers when asked to repeat five numbers. That stood out to the examiner as contrary to her ability as demonstrated in earlier testing and this was a non-credible response pattern, a red flag.

[62] Dr. Bail looked at the plaintiff's test results and concluded that the findings were not consistent with those expected of someone with impaired cognitive functioning.

[63] Dr. Bail concluded that Ms. Baines did not demonstrate the criteria necessary to qualify for any psychiatric disorder. She did not present with an adjustment disorder, a traumatic brain injury, dementia, major depression or anxiety. He believed, therefore, that she would be capable of performing any job that she was reasonably suited to and that she could perform all activities of daily living. In short, she was not psychiatrically impaired.

[64] The fact that the MRI of 25 June 2000 was normal accords with this opinion, he added.

[65] Dr. Upton has been a neurologist since 1968. He met, interviewed and examined Ms. Baines on 16 May 2007. She complained of fatigue, lack of stamina, soreness all over [10 on a scale of 10 or the absolute worst pain imaginable], memory difficulties particularly with short-term memory, inability to concentrate, missing stairs and that she hurt her back while sitting onto a couch.

[66] On sensory testing of touch, vibration, pinprick, pain and temperature, he found inconsistent and variable results. These were red flags to him that the patient may not have been responding in a completely straightforward manner, he said.

[67] He described a seven digit number test which he said was a standardized and validated test in which most people would recall six numbers correctly after 10 min. After 5 min., however, Ms. Baines said she could recall only one number and he said that that was a most unusual result.

[68] Dr. Upton described testing left and right movements. For example when asked to touch the right year with the left finger Ms. Baines got it reversed. He said that that was not likely due to any real problem or that it was not organic, a term which he defined during cross-examination to equal real, correct or truthful..

[69] Dr. Upton was asked to assume that Dr. Mamalek diagnosed a grade 1 concussion and asked whether that was an opinion he agreed with. He said that a grade 1 concussion was the absolute most that Ms. Baines' injury could be considered to involve and, he said, she likely had a less significant injury.

[70] Dr. Upton said that a grade 1 concussion that is not influenced by a subsequent concussion and is left alone gets better. The recovery time for a grade 1 concussion may be hours or days but it should resolve within three months.

[71] Any physical symptoms from soft tissue injuries he would expect to improve with time. As to head injury, Dr. Upton found a number of inconsistent sensory complaints. Ms. Baines told

him that her problem had to do with "connectivity of her neurons". Dr. Upton felt, however, that she had multiple complaints with no clear organic cause and her complaints were out of proportion with her injury. He was not convinced that she suffered any significant impairment or that she should have had work or activities of daily living impairments beyond the few weeks needed to recover from her physical injuries. In his view, she had no significant brain injury.

[72] Dr. Michael Ford is an orthopedic spine and trauma surgeon. Dr. Ford saw Ms. Baines for a defence medical examination on 26 April 2007. His findings on assessing range of motion were entirely normal. He described his assessments as involving simple tests but tests that are sensitive to finding of significant pathology. He found no muscle wasting and no significant pathology.

[73] Dr. Ford's opinion was strictly limited to musculoskeletal issues. He said that at best, Ms. Baines might have had some soft tissue injuries resulting from this accident. He added that, invariably, healing follows the initial inflammatory process for such injuries and typically healing occurs within six weeks.

[74] Dr. Ford stated that there does not seem to be any evidence to require the imposition of restrictions on Ms. Baines' normal activities of daily living, work and recreational included.

[75] Dr. Ford stated repeatedly that the pathophysiology of injury and repair requires that pain coming on much later cannot be causally connected to injury from an accident.

[76] Dr. Ford declined Ms. Baines' invitation to agree that soft tissue injury from a motor vehicle accident like the one in question can come back, on and off, throughout a person's life. Absent ligament damage or fractures, which do not apply to Ms. Baines, injuries do not, in his opinion, produce pain coming and going indefinitely.

[77] The defence consented to Ms Baines filing the reports of Dr Ogilvie-Harris and Dr Kiss. These doctors did not attend to give evidence and were therefore not cross examined. The evidence of Dr Ogilvie-Harris was contradicted by that of Dr Ford. The evidence of Dr Kiss is uncontradicted.

[78] Dr Ogilvie-Harris is an orthopedic surgeon. Ms Baines complained to him of stiffness and pain in her back and shoulders experienced every day, varying with activities. As well, she complained of poor sleep, morning stiffness, poor energy levels and limited endurance.

[79] His physical and neurological examinations were essentially normal although she complained of pain with all movements. He records that he examined her for Waddell's signs. She was positive for tenderness, positive for distraction, positive for simulation, positive for overreaction and positive for regional pain. Five out of five were positive. This indicates an adverse psychological or emotional response to injury and a poor prognosis for recovery, he concluded.

[80] On the basis of the history that Dr. Ogilvie-Harris took, the physical examination he conducted on Ms. Baines and the review of the medical information available to him, he felt that

she did indeed sustain soft tissue injuries to the cervical and lumbar spine as a result of her accident of 26 January 2000. Whether she suffered a head injury is a matter beyond his area of expertise and he offered no opinion on that.

[81] As Ms. Baines continued to complain of pain in the lumbar spine at the time she saw Dr. Ogilvie-Harris in 2005, he concluded that she continued to have generalized musculoskeletal symptoms and that she presented with the characteristic features of a chronic pain syndrome. He felt that her prognosis for recovery from a chronic pain syndrome was guarded and that she would benefit from a chronic pain management program, but it appears that she did not participate in such a program. He concluded that Ms. Baines will have ongoing pain, suffering and limitation of function on a long-term basis that he opined is both serious and permanent.

[82] Dr. Ford stated, and I accept his opinion, that the diagnosis of chronic pain syndrome is controversial and not well understood in the medical community. He said that a psychiatrist might provide a diagnosis of chronic pain syndrome but that it would be impossible for Dr. Ogilvie-Harris to diagnose this syndrome on an objective basis.

[83] Dr Kiss is a neuropsychologist. He performed a neuropsychological assessment and testing on Ms. Baines in 2010, more than 10 years after the motor vehicle accident of January 26, 2000. Dr Kiss did not diagnose or treat Ms Baines.

[84] He reported that the nature of the accident and Ms. Baines' acute phase behaviour suggest that enduring traumatic cerebral involvement is unlikely. He added that Ms. Baines was cooperative and fundamentally effortful but there were repeated indications of likely motivational fluctuations due to emotional factors resulting in performance inconsistencies and likely attenuation in test results. Taking this into account, test results provide insufficient evidence to support neurologically based traumatic cognitive impairment, although fluctuations in performance do suggest variable cognitive efficiency.

[85] He also wrote that while more objective evidence for significant traumatic injury is absent, this accident may have served as a psychological organizing principle, a rationale for pre-existent psychologically-based symptoms and a substrate for the development of additional symptoms. He felt that a clear and evidence-based discussion with Ms Baines soon after the accident regarding the lack of brain injury and early interventions to deal with any emotional reactions may well have prevented the expansion and entrenchment of symptoms.

[86] He opined that while the accident itself was essentially benign, it appears that predisposing psychological factors and unfortunate attitudes promulgated by health professionals have resulted in the clinically significant psychopathology documented during the present assessment. This lady, he said, may benefit from a change in treatment approach, based on an understanding that she has not suffered a significant brain injury and that her current symptoms are largely psychologically-based and likely treatable.

Analysis

[87] Upon the whole of the evidence I find that Ms Baines sustained a concussion in the accident of January 2000. I accept that her concussive injury may have been serious enough to be medically considered as a grade one concussion. The natural course of recovery from such injury involves complete remission of symptoms within about three months. I cannot find that the evidence supports the plaintiff's contention that she suffered a more significant brain injury or that any of the ongoing complaints that she associates with brain injury can reasonably be associated with the concussion she did suffer.

[88] Ms Baines also suffered soft tissue injuries in this accident. I accept the medical evidence given by Dr Ford and find that these physical injuries have resolved and are no longer the source of Ms Baines ongoing complaints.

[89] I do not accept the evidence of Dr Ogilvie-Harris that Ms Baines went on to develop the characteristic features of a chronic pain syndrome. First, whether Dr Ogilvie-Harris is qualified to offer a diagnostic opinion on this matter is not clear from his report and is questioned by Dr Ford. Second, I am not satisfied that Dr Ogilvie-Harris was fair and reasonable in his evidence in accepting Ms Baines' subjective complaints without question in the face of essentially normal physical exam results and findings of positive Waddell's signs, signs positive for distraction, simulation and overreaction. I prefer the evidence of Dr Ford, evidence given in a thorough, concise and balanced fashion and that withstood close cross examination by Ms Baines.

[90] Although Dr Ogilvie-Harris purported to opine that Ms Baines' ongoing subjective complaints and any related limitations are permanent and serious, I reject that evidence. By so saying, Dr Ogilvie-Harris stepped beyond his role as an expert witness and strayed into the domain of the trial judge. By parroting the language of the legislation and applying it to the plaintiff's case, the doctor appears to be intent on influencing me toward an outcome favourable to the plaintiff; that is advocacy and not the impartial, objective or helpful assistance the court expects from an expert.

[91] Further, Dr Ogilvie-Harris attempted to address the ultimate question. That is not acceptable and especially so since he undertook no forensic, thorough or objective analysis of the whole of the circumstances of the plaintiff's complaints in relation to the accident in question. Nor was he present to hear and see the evidence presented in this trial.

[92] It is difficult to know whether the litany of any/all of the complaints of physical and emotional discomforts and limitations detailed in Ms Baines' evidence are honestly felt; for the reasons given by Drs. Bail, Upton, Ogilvie-Harris and Ford, whose examination findings appear to cast doubt or raise red flags of concern regarding the accuracy and completeness of her subjective complaints and because of the inconsistencies within and between the descriptions of the plaintiff's complaints as detailed by witnesses at this trial.

[93] It must be noted that Ms Baines comported herself throughout this trial in a composed, respectful, dignified and reasonably efficient manner. Advocacy in a courtroom setting is a complicated art that challenges even trained and experienced lawyers. Nevertheless, Ms Baines

accomplished the presentation of her case very well. She demonstrated cognitive functioning at a high level. In this lengthy trial, Ms Baines demonstrated none of the limitations of which she has complained during the many years since the accident.

[94] My observations of Ms Baines during this litigation mirror those recorded by Dr Kiss, who wrote that Ms. Baines was alert, oriented and responsive during interviews and assessment. There was no evidence of somnolence, sedation, hesitancy or psychomotor slowing. She was socially appropriate and pleasant. She was able to produce fairly detailed accounts and spoke in a reasonably articulate manner. She did require repetition of questions and simplification of instructions at times. She was not overly prone to distraction or fatigue. There were no observed lapses of attention or indications of significant memory problems. Vision, hearing and comprehension of test instructions were sufficient for purposes of assessment.

[95] The stamina, energy and abilities demonstrated in the course of this litigation, the fact that Ms Baines is actively pursuing three other cases as well and the red flags referred to by the medical experts are very troubling and cast a long and dark shadow of doubt over the accuracy and completeness of the evidence given by Ms Baines and the lay witnesses she called.

[96] In the result, I cannot accept that the plaintiff's self description of subjective complaints and limitations is fair or accurate. She has not met the burden upon her for she has not favoured the court with credible, cogent, consistent evidence.

[97] Another important question here is whether or to what extent Ms Baines' complaints and limitations have been caused or contributed to by the accident in question. Ms Baines bears the onus of establishing that the impairments she alleges exist and are permanent impairments.¹ Clearly the causal connection, if it exists, cannot be demonstrated by the opinion of the plaintiff herself, no matter how firmly held that opinion may be. Medical science may have an answer; specialists in psychiatry and/or psychology may be able to answer this question but, in this trial, none have. Dr Bail answered the question but in the negative by concluding that she presented with no diagnosable psychiatric disorder at all when he saw her in November of 2005.

[98] Dr Mamelak has treated Ms Baines for over a decade but he was not asked to provide an updated report containing his diagnoses and prognoses and so he could not, nor did he, speak to such matters from his report authored in 2002.

[99] In my view, Dr Kiss came closest of all the medical experts in this case to explaining Ms Baines' situation. He speaks of emotional reactions expanding over time and becoming entrenched symptoms following upon an accident that was essentially benign, in concert with predisposing psychological factors. I accept his opinion that Ms Baines has not suffered a significant brain injury and that her current symptoms are largely psychologically based and likely treatable.

¹ *Bisier v Thorimbert*, [2006] O.J. No. 4026 (S.C.J.)

The Law

[100] The issue before this court is whether the plaintiff has sustained impaired functioning as a result of her accident so as to overcome the prohibition against suing the driver and owner under Bill 59 enacted under the Insurance Act. Bill 59 was applicable to motor vehicle accidents which occurred in Ontario after November 1, 1996 and at the time of the accident in question, in January 2000.

[101] The statutory bar in effect at the time of the subject motor vehicle accident provided an immunity of the “at fault” party from liability unless the plaintiff could satisfy the court on a balance of probabilities that she has sustained...

- (a) Permanent serious disfigurement; or
- (b) Permanent serious impairment of an important physical, mental or psychological function.

The issue on this motion deals with part (b) of this exemption.²

[102] The leading authority on the interpretation of the “threshold” under Bill 59 is *Meyer v. Bright* 1993 CanLII 3389 (ON C.A.), (1993), 15 O. R. (3d) 129 (C.A.). There, the court articulated the three questions which must be posed when considering whether a plaintiff has fallen within the ambit of one of the statutory section relevant to this action. Those questions are:

- a. Has the injured person sustained a permanent impairment of a physical, mental or psychological function?
- b. If yes, is the function one which is permanently impaired an important one?
- c. If yes, is the impairment of the important function serious?³

[103] Clearly, Ms Baines is convinced that she suffered a permanent impairment of the physical functioning of her brain. She is persuaded that when she bumped her head during the motor vehicle accident of 26 January 2000, she sustained a brain injury with resultant cognitive and sensory symptoms and limitations as described above.

[104] In the alternative, Ms. Baines asserts that her physical, mental and/or psychological functioning has been impaired as the result of whatever injuries she sustained in this accident.

² *Kayhan v. Greve*, [2011] ONSC 872 (CanLII) at paras. 4 & 5

³ *Ibid* at para 24

[105] The medical evidence that I accept and credit in this case does not establish that Ms Baines has, on the basis of a balance of probabilities, suffered any permanent impairment of a physical function. The alleged chronicity of certain complaints notwithstanding, I accept the evidence of Drs Bail, Upton and Ford and find that Ms Baines has long since recovered from her concussive and soft tissue injuries, those being the only physical injuries alleged.

[106] As stated above, I cannot take as proven through the evidence of Dr Mamelak that the plaintiff has suffered a permanent impairment of a mental or psychiatrically diagnosable function as the result of this accident. In any event, I accept the evidence of Dr Bail and find that Ms Baines has not suffered any ongoing impairment of a mental function at all.

[107] The plaintiff has not established a permanent impairment of a psychological function. Whatever psychological issues Dr Kiss suggested may affect Ms Baines have not been diagnosed nor proven to have been caused by injuries suffered in the accident in question. Further, Dr Kiss has determined that Ms Baines' psychological situation is likely treatable and, as such, the available evidence does not establish that the condition is permanent.

[108] These findings bring an end to my analysis of the relevant statutory exemptions.

[109] Therefore, the defendants' motion succeeds. The plaintiff's claims for non-pecuniary damages must be dismissed.

[110] Although the jury awarded an amount for past income loss, the parties agree that that amount is lower than is the amount of income replacement benefits paid to Ms Baines by her own auto insurer in 2000 and, therefore, her net recoverable pecuniary loss is zero. The result of this motion and the impact of the deduction of accident benefits taken together with the answers made by the jury to questions concerning the remaining heads of damages claimed at trial is that the defendants are protected against or otherwise not liable to pay anything to Ms Baines against any of her damages claims in this action.

[111] If the parties cannot resolve costs issues, I may be spoken to.

Moore J.

DATE: 23 March 2011

